A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

17 April 2026

Dr Phillip Boltin
Betta Health Medical Centre

Dear Dr Boltin,

Re: Ms Marree Brown
DOB: 1966-03-19
Address: 82 Brunswick Road, BRUNSWICK, VIC, 3056

Summary

- **Progressive iron deficiency despite oral supplementation.**
- **For gastroscopy and concurrent iron infusion.**
- **Consideration of capsule endoscopy if gastroscopy is non-diagnostic.**
- **No action required by you.**

I had the pleasure of reviewing Marree Brown regarding her progressive iron deficiency. As you have noted, her ferritin levels have trended downwards from 150 µg/L to 18 µg/L over the last two years. This decline occurred despite a three-month trial of consistent oral iron supplementation, which suggests either an ongoing source of occult blood loss or a malabsorptive process.

Clinically, Marree reports recent lethargy but remains physically active. She denies any overt symptoms of gastrointestinal blood loss, such as haematemesis, melaena, or haematochezia. There have been no symptoms of haematuria or unintentional weight loss. Her dietary red meat intake is modest, at approximately once per week. We discussed potential contributing factors, including a history of significant gingival bleeding over the past two years related to periodontal disease, which has recently been managed with debridement. She also noted a brief recent course of non-steroidal anti-inflammatory drugs (NSAIDs) for back pain, though this was not a chronic requirement.

Impression and Plan Given that her iron levels have failed to respond to oral replacement, further investigation of the upper gastrointestinal tract is warranted. A colonoscopy performed last year was unremarkable and does not need to be repeated at this stage. I have recommended a

All appointments and enquires
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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gastroscopy to exclude coeliac disease or other sources of malabsorption and occult bleeding.

Marree has expressed a preference to have this performed in the public system to avoid private health insurance excesses. Consequently, I will facilitate a referral to St Vincent's Hospital for the procedure. I have also recommended that she receive an intravenous iron infusion at the time of her gastroscopy. If the gastroscopy is non-diagnostic, the next step in our investigation will be a capsule endoscopy to evaluate the small bowel. I have advised Marree to continue her oral iron supplements in the interim and to contact my rooms if she has not received a procedure date

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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